LABORATORY REPORT

Name	: **Mrs. SHWETAL SHAH**	Sex/Age	: **Female / 1 Year 11 Months**	Case ID	: **21200102716**
Ref. By	: Dr. Atul P Munshi MD DGO	Dis. At	:	Pt. ID	: 402375
Bill. Loc.	:			Pt. Loc	:

Reg Date and Time	: 03-Dec-2022 07:46	Sample Type	:	Mobile No.	: 7819811953
Sample Date and Time	: 03-Dec-2022 07:46	Sample Coll. By	: NSRL	Ref Id1	:
Report Date and Time	:	Acc. Remarks		Ref Id2	:

Abnormal Result(s) Summary

Test Name	Result Value	Unit	Reference Range
Haemogram (CBC)			
Lymphocyte	**18.1**	%	20.00 - 40.00
Eosinophil	**6.3**	%	1.00 - 6.00
Lymphocyte	**1808**	/ul	6000 - 9000
Neutrophil to Lymphocyte Ratio (NLR)	**3.86**		0.78 - 3.53
ESR	**33**	mm after 1hr	3 - 20

Abnormal Result(s) Summary End

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

Neuberg Supratech Reference Laboratories Private Limited
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Bill. Loc.	:			Pt. Loc	:
Reg Date and Time	: 03-Dec-2022 07:46	Sample Type	: Whole Blood EDTA	Mobile No.	: 7819811953
Sample Date and Time	: 03-Dec-2022 07:46	Sample Coll. By	: NSRL	Ref Id1	:
Report Date and Time	: 03-Dec-2022 08:48	Acc. Remarks		Ref Id2	:

HAEMOGRAM REPORT

TEST	RESULTS	UNIT	BIOLOGICAL REF. INTERVAL	REMARKS
<u>HB AND INDICES</u>				
Haemoglobin (Colorimetric)	11.6	G%	11.10 - 14.10	
RBC (Electrical Impedance)	4.06	millions/cumm	4.00 - 5.20	
PCV(Calc)	34.10	%	30.00 - 38.00	
MCV (RBC histogram)	84.0	fL	75.00 - 87.00	
MCH (Calc)	28.6	pg	24.00 - 30.00	
MCHC (Calc)	34.0	gm/dL	31.00 - 37.00	
RDW (RBC histogram)	14.30	%	11.00 - 16.00	
<u>TOTAL AND DIFFERENTIAL WBC COUNT (Flowcytometry)</u>				
Total WBC Count	9990	/μL	5000.00 - 15000.00	

		[%]		EXPECTED VALUES		[Abs]		EXPECTED VALUES
Neutrophil		69.9	%	40.00 - 70.00		6983	/ul	1500.00 - 8000.00
Lymphocyte	L	**18.1**	%	20.00 - 40.00	L	**1808**	/ul	6000 - 9000
Eosinophil	H	**6.3**	%	1.00 - 6.00		629	/ul	100.00 - 1000.00
Monocytes		5.5	%	2.00 - 10.00		549	/ul	200.00 - 1000.00
Basophil		0.2	%	0.00 - 2.00		20	/ul	0.00 - 100.00

TEST		RESULTS	UNIT	BIOLOGICAL REF. INTERVAL
Neutrophil to Lymphocyte Ratio (NLR)	H	**3.86**		0.78 - 3.53
<u>PLATELET COUNT (Optical)</u>				
Platelet Count		394000	/μL	200000.00 - 500000.00
MPV		9.50	fL	6.5 - 12
PDW		15.6		15.0 - 17.0
<u>SMEAR STUDY</u>				
RBC Morphology		Normocytic Normochromic RBCs.		
WBC Morphology		Total WBC count within normal limits.		

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Dr. Aakash Shah
M.D. (Pathologist)

Dr. Sandip Shah
M.D. (Path. & Bact.)
Consultant Pathologist

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Platelet	Platelets are adequate in number.
Parasite	Malarial Parasite not seen on smear.

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TEST		RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
HAEMATOLOGY INVESTIGATIONS					
ESR *Photometrical capillary stopped flow kinetic analysis*	H	**33**	mm after 1hr	3 - 20	

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Reg Date and Time	: 03-Dec-2022 07:46	Sample Type	: Serum	Mobile No.	: 7819811953
Sample Date and Time	: 03-Dec-2022 07:46	Sample Coll. By	: NSRL	Ref Id1	:
Report Date and Time	: 03-Dec-2022 10:15	Acc. Remarks		Ref Id2	:

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
BIOCHEMICAL INVESTIGATIONS				
Follicle Stimulating Hormone *CMIA*	**9.85**	mIU/mL	2.5-10.2 Follicular phase 3.4-33.4 Mid cycle peak 1.5-9.1 Luteal phase <0.3 Pregnant 23.0-116.3 Post Menopausal	

INTERPRETATIONS:

Useful for an adjunct in the evaluation of menstrual irregularities, Evaluating patients with suspected hypogonadism , Predicting ovulation , Evaluating infertility & Diagnosing pituitary disorders.

In both males and females, primary hypogonadism results in an elevation of basal follicle-stimulating hormone (FSH) and luteinizing hormone (LH) levels.

FSH and LH are generally elevated in:

-Primary gonadal failure , Complete testicular feminization syndrome, Precocious puberty (either idiopathic or secondary to a central nervous system lesion), Menopause (postmenopausal FSH levels are generally >40 IU/L), Primary ovarian hypofunction in females , Primary hypogonadism in males.

Normal or decreased FSH in:

-Polycystic ovary disease in females

FSH and LH are both decreased in failure of the pituitary or hypothalamus.

CAUTIONS:

Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedures, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
BIOCHEMICAL INVESTIGATIONS				
Leutinizing Hormone *CMIA*	**3.06**	mIU/mL	Follicular phase 2.39 to 6.6 mIU/mL Mid cycle peak 9.06 to 74.24 mIU/mL Leuteal phase 0.9 to 9.33 mIU/mL Post menopausal 10.39 to 64.57 mIU/mL	

INTERPRETATIONS:

Useful as an adjunct in the evaluation of menstrual irregularities, Evaluating patients with suspected hypogonadism, Predicting ovulation, Evaluating infertility, Diagnosing pituitary disorders.

In both males and females, primary hypogonadism results in an elevation of basal follicle-stimulating hormone (FSH) and luteinizing hormone (LH) levels.

Postmenopausal LH levels are generally >40 IU/L. (Note: FSH is the preferred test to confirm menopausal status.)

FSH and LH are generally elevated in:
Primary gonadal failure, Complete testicular feminization syndrome, Precocious puberty (either idiopathic or secondary to a central nervous system lesion), Menopause, Primary ovarian hypodysfunction in females, Polycystic ovary disease in females, Primary hypogonadism in males

LH is decreased in Primary ovarian hyperfunction in females , Primary hypergonadism in males.

FSH and LH are both decreased in failure of the pituitary or hypothalamus

CAUTIONS:

No clinically significant cross-reactivity has been demonstrated with follicle-stimulating hormone, thyroid stimulating hormone, or human chorionic gonadotropin.
Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedures, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results

----------------- End Of Report -----------------

For test performed on specimens received or collected from non-NSRL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NSRL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

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